

Example 1

1 (a) Metastatic cancer of lung (adenocarcinoma) [2C25.0](#)

(b)

(c)

(d)

2

Adenocarcinoma can be primary in lung. Lung is the only site mentioned or implied on the certificate. Code as primary malignant neoplasm of lung at Adenocarcinoma of bronchus or lung (2C25.0).

If the certificate mentions several malignant neoplasms that are on the list of common sites of metastases and one or more of them are specified as 'metastatic', then code all of them as secondary malignant neoplasms. Also add a code for unspecified primary malignant neoplasm [2D4Z](#).

- *exception:* Code a 'metastatic neoplasm of lung' as primary malignant neoplasm of the lung Malignant neoplasms of bronchus or lung, unspecified (2C25.Z) if all other neoplasm sites reported on the death certificate are on the list of common sites of metastases, whether they are described as 'metastatic' or not.
- *exception:* For 'metastatic' neoplasms of a specified histopathology' which are on the list of common sites of metastases, see subsection '**Metastatic malignant neoplasm of a specific histopathology**' above.

'Metastatic' malignant neoplasm not on the list of common sites of metastases

If the certificate mentions a single malignant neoplasm, and this neoplasm is not on the list of common sites of metastases but it is specified as 'metastatic', then code as primary malignant neoplasm of that particular site.

If the certificate mentions several malignant neoplasms that are not on the list of common sites of metastases and all of them are specified as 'metastatic', then code all neoplasms as primary.

If the certificate mentions several malignant neoplasms, and none of them is on the list of common sites of metastases and some but not all are specified as 'metastatic', then code a neoplasm not specified as 'metastatic' as primary and a neoplasm specified as 'metastatic' as secondary.

See Section [2.21.5.2 Malignant neoplasms: primary or secondary?](#) for blocks used for primary or secondary.

'Metastatic' malignant neoplasm, some on the list of common sites of metastases and some not

If the certificate mentions several malignant neoplasms and some but not all are on the list of common sites of metastases and some but not all are specified as 'metastatic', then code a neoplasm on the list of common sites of metastases as secondary ([2D50-2E2Z](#)). Also, code a neoplasm not on the list of common sites of metastases and specified as 'metastatic' as secondary, and a neoplasm not on the list of common sites of metastases and not specified

as 'metastatic' as primary (See Section [2.21.5.2](#) for the blocks used for primary malignant neoplasms).

- *exception:* Code neoplasms, even if described as 'metastatic', as primary when reported as due to a condition that increases the risk of a malignant neoplasm of that site or tissue, whether the site is on the list of common sites of metastases or not.

Example 1

- 1 (a) Liver cancer [2D80.0](#)
(b) Metastatic colon cancer [2B90.Z]
(c)
(d)
- 2

Code as Malignant neoplasm metastasis in liver (2D80.0) and Malignant neoplasms of colon, unspecified (2B90.Z) . Liver is on the list of common sites of metastases but colon is not.

Example 2

- 1 (a) Metastatic gallbladder cancer [2C13.Z](#)
(b) Metastatic colon cancer [2B90.Z](#)
(c)
(d)
- 2

Code both Malignant neoplasms of gallbladder, unspecified (2C13.Z) and Malignant neoplasms of colon, unspecified (2B90.Z) as primary. The order of entry does not affect the coding.

2.21.6 Sequelae

A sequela is a chronic condition resulting from an acute condition and which begins during that acute condition. The acute condition is itself no longer present.

The classification provides certain categories to be used when conditions are reported as sequelae, late effects, or other conditions specified in this section (e.g. [1G80-1G8Z Sequelae of infectious diseases](#)). Where no specific category is provided for the condition described as a sequela (e.g. late effects of injuries are coded to the residual category of the chapter that may also include acute conditions), use additional code [XT9C](#) Cause of late effect, if desired, to identify that the first condition was reported as a cause of a sequela condition (e.g. Head injury sequelae: NA0Z&XT9C Injuries to the head, unspecified & Cause of late effect).

Note: When deciding whether a condition is a sequela or late effect, the stated duration of all subsequent conditions should be taken into account.

Example 1	Duration
1	(a) Aspiration pneumonia
	(b) Dysphagia
	(c) Stroke
	(d)
2	

Code stroke to the late effect of stroke, as it has caused something with a duration longer than the required time for stroke to be considered late effect.

2.21.6.1 Conditions considered to be sequelae

Consider the following categories present one year or more after onset of the previous condition as a late effect:

- [1G84](#) Sequelae of viral encephalitis
- [1G85](#) Sequelae of diphtheria
- [1G8Y](#) Sequelae of other specified infectious diseases
- [5B63](#) Sequelae of rickets
- [8B25](#) Late effects of cerebrovascular disease
- Late effects of injuries, of poisoning or of certain other consequences of external causes
- Late effects of external causes of morbidity or mortality

2.21.6.2 Sequelae of tuberculosis

Code tuberculosis ([1B10.0](#) - [1B10-1B1Z](#)) to sequelae of tuberculosis ([1G80](#)) if the condition is specified as such or as arrested, cured, healed, inactive, old or quiescent, or similar descriptions unless there is evidence of active tuberculosis. This does not include chronic tuberculosis, which should be coded as active infectious disease.

2.21.6.3 Sequelae of trachoma

Code trachoma ([1C23](#)) to sequelae of trachoma ([1G81](#)) if trachoma is specified as healed or inactive and certain specified sequelae, such as blindness, cicatricial entropion and conjunctival scars, unless there is evidence of active infection. It does not include chronic trachoma, which should be coded as active infectious disease.

2.21.6.4 Sequelae of viral encephalitis, diphtheria or other specified infectious diseases

Condition	Active Codes	Sequelae Code
Viral encephalitis	1C80 , 1C83 – 1C8D	1G84
Diphtheria	1C17	1G85
Other specified infectious disease, except for acute rheumatic fever, and HIV	1A00 - 1A9Z , 1B21 - 1B2Z , 1B50 - 1C16 , 1C18 - 1C22 , 1C2Y - 1C2Z , 1C82 , 1C8E , 1C8F , 1C8Y , 1G80	1G8Y

Code the above infectious disease to the appropriate sequelae code of the infectious condition if the condition is specified as such or as arrested, cured, healed, inactive, old or quiescent. Sequelae also include conditions present one year or more after onset of conditions classifiable to infectious disease categories, unless there is evidence of active disease. This does not include chronic infectious diseases, which should be coded as active infectious disease.

2.21.6.5 Sequelae of malnutrition or certain specified nutritional deficiencies

Condition	Active Codes	Sequelae Code
Protein energy malnutrition	5B50 - 5B54 , 5B71	5B60
Vitamin A deficiency	5B55	5B61
Vitamin C deficiency	5B56	5B62
Rickets	5B57.0	5B63
Other specified nutritional deficiencies	5B57.1 – 5B5K , 5B70 , 5B7Y	5B6Y
Unspecified nutritional deficiencies	5B7Z	5B6Z

Code the above malnutrition or certain specified nutritional deficiencies to the appropriate sequelae code of malnutrition or certain specified nutritional deficiencies if the condition is stated to be a sequela or late effect of or as the cause of conditions present one year or more after onset of the condition. This does not include chronic malnutrition or nutritional deficiency, which should be coded to current malnutrition or nutritional deficiency.

2.21.6.6 Late effects of Chapter 22 and Chapter 23

Code any injury, poisoning or certain other consequences of external causes (Chapter 22) or external causes of morbidity or mortality (Chapter 23) to late effects of injury, poisoning or certain other consequences of external causes or late effects of external causes of morbidity or mortality include those specified as such, or as sequelae, and those present one year or more after the acute injury or originating event. Code to the originating active condition or external cause and use additional code [XT9C](#) if desired to the cluster to retain the information that it was reported as a late effect.

2.21.6.7 Sequelae of leprosy

Code leprosy ([1B20](#)) to sequelae of leprosy if leprosy is stated to be a sequela or late effect of leprosy, or if a chronic condition or a condition with a duration of longer than a year that is reported due to leprosy is reported.

2.21.6.8 Sequelae of poliomyelitis

Code poliomyelitis ([1C81](#)) to sequelae of poliomyelitis ([1G83](#)):

- if poliomyelitis is specified as such or as history (of) or old, or duration of onset is more than a year

or

- if a chronic condition or a condition with a duration of longer than a year that is reported due to poliomyelitis is reported.

2.21.7 Consistency between sex of patient and diagnosis

Most categories of ICD–11 apply to all persons without reference to sex. However, some diseases are more likely to occur in one biological sex. A list of those conditions is given in:

- [3.14 Annex C: Annexes for Mortality Coding](#)
- [3.14.11 List of categories limited to, or more likely to occur in, female persons](#)
- [3.14.12 List of categories limited to, or more likely to occur in, male persons](#)

When addressing cases of inconsistency between sex of patient and diagnosis reported, it is important to be aware of issues around gender identity. National legislation may regulate the recognition of gender identity or gender reassignment (sex-change) including protection of privacy. If there are special obligations to respect confidentiality around gender reassignment cases, these must be taken into account in the coding decisions and the onward dissemination of the coded data.

The general recommendation for handling the situation where there is an apparent inconsistency between sex and diagnosis reported follows. However, this is a general guideline which may not always be applicable because legal requirements vary among countries. It is recommended to manually check all cases of inconsistencies between sex of patient and diagnosis reported. When data processed automatically in line with guidelines below, these cases should be flagged for additional manual check.

If there might be an inconsistency between the sex of the deceased and the cause of death reported:

- check the information and make sure that no reporting error occurred. Further information may be available from the certifier or registration officials. If checking shows that either sex or diagnosis is an error, correct the one which is believed to be wrong.

If no further information is available and:

- it cannot be decided whether there is an error, or which of the two data items (sex and diagnosis) is the one that is wrong, retain the recorded sex and code to [MH14](#) Other ill- defined and unspecified causes of mortality*.
- it appears that a gender reassignment case or similar situation is involved, retain the recorded sex and code if possible, to a substitute code which is similar to the reported diagnosis but not specific to either sex. For example, a neoplasm of sex-specific genital organs could be coded to [2D4Y](#) Other specified malignant neoplasms

of unspecified primary sites. If no suitable substitute code exists, code to [MH14](#) Other ill- defined and unspecified causes of mortality*.

*Consider adding a note to the statistics, specifying the number of cases recoded because of apparent inconsistencies between sex and cause, while taking into consideration confidentiality required in each country.

2.21.8 Specific instructions on other ICD categories

2.21.8.1 Acute or chronic rheumatic heart diseases

Rheumatic heart diseases are classified to Acute rheumatic fever with heart involvement (1B41) or to chronic conditions at fifth character 0 of *BB60-BC0Z Heart valve diseases*, or [BC20](#) Chronic rheumatic heart diseases, not elsewhere classified, *depending on whether the rheumatic process being described is active or inactive. If there is no statement that the rheumatic process was active or inactive at the time of death, code the following cardiac conditions as active* ([1B41](#) Acute rheumatic fever with heart involvement):

- a cardiac condition reported as due to rheumatic fever, except cardiac arrest, acute heart failure, bacterial endocarditis;
- a cardiac condition specified as rheumatic and described as acute or subacute;
- carditis, endocarditis, heart disease, myocarditis or pancarditis, described as rheumatic or reported as due to a rheumatic disease, and the duration is less than one year;
- carditis, endocarditis, heart disease, myocarditis or pancarditis, described as rheumatic or reported as due to a rheumatic disease, and the deceased is less than 15 years old.

2.21.8.2 Obstetric death of unspecified cause, Obstetric deaths 42 days–1 year after delivery, sequelae of obstetric causes

The International form of medical certificate of cause of death (See Mortality Annex [3.14](#) is structured to allow reporting on obstetric causes, the time elapsed between the obstetric event and the person's death, and whether the pregnancy contributed to death. Use all information available on the death certificate. When information provided is ambiguous it is recommended to verify where possible, while means of verification may vary among countries according to different legal systems or profiles in maternal mortality. Additional information may be obtained through clinical summaries of medical institutions, verbal autopsy reports, or by certain verification processes which may require not only queries to the certifier but also establishing an inquiry system to analyse specific cases.

Following concepts related to statistical tabulation of maternal mortality is provided in Section [2.25.5](#) Standards and reporting requirements related for maternal mortality.

- [2.25.5.1](#) Maternal death
- [2.25.5.2](#) Late Maternal death
- [2.25.5.3](#) Comprehensive maternal death
- [2.25.5.4](#) Direct and indirect obstetric deaths
- [2.25.5.5](#) Death occurring during pregnancy, childbirth and puerperium
- [2.25.5.6](#) Recording requirements of maternal mortality
- [2.25.5.7](#) International reporting of maternal mortality
- [2.25.5.8](#) Numerator, denominator, and ratios of published maternal mortality

Briefly the underlying cause of death categories for Maternal Mortality is summarized in the table below:

	JB00 - JB60 , JB63 .-, JB64 .-, JB6Y , 1C14 Maternal death	JB61 .- Late Maternal death	JB62 .- Sequelae of obstetric conditions
Decesed was pregnant:	at the time of death, and within 42 days before the death	more than 42 days but less than one year before the death	one year or more before the death

On a death certificate usually the timespan is recorded in day unit where 42 days are included in maternal death and 43 days are included in late maternal death. In a mathematical explanation, this means exactly 42 days are included in maternal death, while for example 42 days and one hour is included in late maternal death. Also note that [JB61](#).- and [JB62](#).- includes deaths due to any obstetric cause. The obstetric cause reported including cause unknown ([JB60](#)) is postcoordinated to [JB61](#).- or [JB62](#).- to retain information on the cause. Coding instructions are provided to select the UCOD and capture further details in a cluster.

Coding instructions for maternal mortality

For coding of maternal mortality, follow the general coding instructions.

To assign the correct multiple cause code for a certificate with mention of pregnancy, first use the coding tool to assign a specific code for each condition reported. Categories out of Chapter 18 may also be assigned. When a condition suggests it is an obstetric condition, by using certain modifiers such as 'obstetric' or 'maternal', search if there is a direct match of the diagnosis reported. The coding tool also supports coding by providing the icon 'J' () for certain condition that have a compatible category for maternal conditions.

Example 1

- 1 (a) Pulmonary oedema [CB01](#)
(b) Mitral valve insufficiency, pregnancy [JB64.4/BB61.Z](#)
(c)
(d)

2

Mitral valve insufficiency nos is coded to [BB61.Z](#), however assign [JB64.4/BB61.Z](#) because it is described as 'pregnancy'.

Example 2

- 1 (a) Pulmonary oedema [CB01](#)
(b) Mitral valve insufficiency [BB61.Z](#)
(c)
(d)

2 XX completed weeks of gestation

Code mitral valve insufficiency to [BB61.Z](#). Pregnancy is mentioned in Part 2 and it is considered that pregnancy contributed to death. Apply Step M4 and code the underlying cause of death to [JB64.4](#) Diseases of the circulatory system complicating pregnancy, childbirth or the puerperium. For greater specificity, also add the code for [BB61.Z](#) Mitral valve insufficiency, unspecified to the cluster ([JB64.4/BB61.Z](#)).

Structure of Chapter 18 and other related categories

The following categories are used for deaths due to an obstetric event occurring within 42 days after termination of pregnancy:

- [JA00](#) - [JB4Z](#), [1C14](#): Direct obstetric causes
- [JB63.-](#), [JB64.-](#): Maternal diseases classifiable elsewhere but complicating pregnancy
- [JB60](#) is used when a woman dies during pregnancy, labor, delivery or the puerperium and the only information provided is 'maternal' or 'obstetric' death. If obstetric cause of death is specified, do not use [JB60](#) but code to the appropriate category.
- [JB6Y](#) is used for other specified obstetric conditions not elsewhere classified

Category [JB61.-](#) is used for death of a woman due to an obstetric cause but more than 42 days but less than one year after termination of pregnancy.

Category [JB62.-](#) is used for death of a woman due to an obstetric cause of one year or more after termination of pregnancy.

[JB6Z](#) is used when both obstetric condition is unspecified, and the time elapsed between the obstetric event and death is unknown. Note that this code is not to be used for underlying cause of death (See section [2.19.4](#) Special instructions on surgery and other medical procedures (Step M4)).

Determining whether pregnancy contributed to death

Consider pregnancy contribute to death when pregnancy, puerperium or childbirth is reported in Part 1 or Part 2; or is reported elsewhere and the answer to the question “Did the pregnancy contribute to death?” is yes, unknown, or is unstated.

Determining the time elapsed

The time elapsed between the obstetric event and the death is determined by the duration reported for the obstetric cause. If duration is unknown or unspecified, use the information in Frame B of the death certificate. When duration is unknown or unstated, but pregnancy contributed to death, it is assumed that the death occurred within 42 days after the obstetric event.

2.21.8.3 Deaths due to Certain conditions originating in the perinatal period

Chapter 19 ‘Certain conditions originating in the perinatal period’ includes conditions that have their origin in the perinatal period even though death or morbidity occurs later.

For decedents less than 28 days old, assume that a reported condition developed in the perinatal period, unless the duration is stated, and the onset was after the first completed week of life.

Use a code from Chapter 19, if:

- the condition is included in Chapter 19.
- the condition is specified as congenital/perinatal/newborn.
- the duration of the condition indicates that the condition developed in the neonatal or perinatal period. This applies even if the condition is not specified as neonatal or perinatal on the certificate.

Some conditions originating in the perinatal period are excluded from Chapter 19, such as:

- Intestinal infectious diseases ([1A00-1A40.Z](#))
- Congenital syphilis ([1A60](#))
- Congenital gonococcal infection ([1A70-1A70-1A7Z](#))
- Tetanus neonatorum ([1C15](#))
- HIV disease ([1C60-1C62.Z](#))
- Infectious diseases([1A00](#)-[1H0Z]), acquired after birth
- Neoplasms ([2A00-2F9Z](#))
- Hereditary haemolytic anaemia ([3A10](#))
- Transient hypogammaglobulinaemia of infancy ([4A01.03](#))
- Endocrine, nutritional and metabolic diseases ([5A00-5D46](#))
- Certain congenital diseases of the nervous system ([8A00-8E7Z](#))
- Endocardial fibroelastosis ([BC43.3](#))
- Intestinal obstruction or paralytic ileus ([DA93.0](#))
- Pemphigus neonatorum and Staphylococcal scalded skin syndrome ([EA50](#))
- Cradle cap ([EH40.00](#))
- Diaper (napkin) dermatitis ([EH40.10](#))
- Developmental anomalies ([LA00-LD9Z](#))
- Injury, poisoning and certain other consequences of external causes ([NA00-NF2Z](#))

For some conditions diagnosed under a specific age, it is assumed that the condition was congenital. See the following section, 'Developmental anomalies'.

Fetus or newborn affected by maternal factors or by complications of pregnancy, labour or delivery

Conditions of the mother affecting the fetus or newborn are to be reported in Part 1 or Part 2 of the death certificate and should, where possible, be coded to [KA00-KA0Z](#) Fetus or newborn affected by maternal factors or by complications of pregnancy, labour or delivery.

When a specified maternal condition affecting the fetus or newborn is initially coded outside Chapter 19 by the coding tool, code to [KA00-KA0Z](#). Also use additional code if desired to identify the specific maternal condition reported.

Consider a condition as maternal if it is specified as such, or if the term itself indicates it is a maternal condition (e.g. obstetric haemorrhage), or additional information reported in Frame B: Other medical data, suggests the condition is maternal.

Example 1

- 1 (a) Prematurity [KA21.4Z](#)
- (b) Cervical incompetence [KA01.0](#)
- (c)
- (d)
- 2

The maternal condition, cervical incompetence, affected the newborn by causing prematurity. The code provided by the coding tool is [GA15.6](#) which is outside Chapter 19. However, there is an exclusion note for

fetus or newborn affected by incompetence of cervix uteri. So, code to [KA01.0](#) Fetus or newborn affected by incompetence of cervix uteri.

The following table provides codes for certain maternal conditions and the corresponding codes for these conditions when affecting the fetus or newborn. Note that there are conditions not listed but are still coded to [KA00-KA0Z](#). For example, maternal chemotherapy or surgical procedure on mother may not be assigned a code but still can be classified to [KA00.9](#) and [KA00.A](#) respectively.

Maternal conditions

[JA20-JA2Z](#) Oedema, proteinuria, or hypertensive disorders in pregnancy, childbirth, or the puerperium, except [JA22](#)

[JA22](#) Gestational oedema or proteinuria without hypertension

[GB40-GC2Z](#) Diseases of the urinary system

Chapter 01 Certain infectious or parasitic diseases

[DA0C](#) Periodontal disease

Chapter 12 Diseases of the respiratory system, or [LA70-LA7Z](#) Structural developmental anomalies of the respiratory system

[5B50-5C3Z](#) Nutritional disorders

Chapter 22 Injury, poisoning or certain other consequences of external causes

[JB64.0](#) Anaemia complicating pregnancy, childbirth or the puerperium

[GA15.6](#) Incompetence of cervix uteri

[JA01](#) Ectopic pregnancy

[JA40-JA4Z](#) Obstetric haemorrhage

[JA80](#) Maternal care related to multiple gestation

[JA87](#) Maternal care related to polyhydramnios

[JA88.0](#) Oligohydramnios

[JA89](#) Maternal care related to premature rupture of membranes

[JA8A](#) Maternal care related to placental disorders - [JA8C](#) Maternal care related to premature separation of placenta

[JB08](#) Labour or delivery complicated by umbilical cord complications

Code to:

[KA00.0](#) Fetus or newborn affected by maternal hypertensive disorders

[KA00.1](#) Fetus or newborn affected by gestational oedema or proteinuria without hypertension

[KA00.2](#) Fetus or newborn affected by maternal renal or urinary tract diseases

[KA00.3](#) Fetus or newborn affected by maternal infectious diseases

[KA00.4](#) Fetus or newborn affected by periodontal disease in mother

[KA00.5](#) Fetus or newborn affected by maternal respiratory diseases

[KA00.6](#) Fetus or newborn affected by maternal nutritional disorders

[KA00.8](#) Fetus or newborn affected by maternal injury

[KA00.B](#) Fetus or newborn affected by maternal anaemia

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA01](#) Fetus or newborn affected by maternal complications of pregnancy

[KA02](#) Fetus or newborn affected by complications of placenta

[KA03](#) Fetus or newborn affected by complications of umbilical cord

Maternal conditions

[JA88](#) *Maternal care related to certain specified disorders of amniotic fluid or membranes, except*
[JA88.0](#)

[JB00-JB0Z](#) *Complications of labour or delivery*

Code to:

[KA04](#) *Fetus or newborn affected by other abnormalities of membranes*

[KA05](#) *Fetus or newborn affected by certain complications of labour or delivery*

2.21.8.4 Special instructions on fetal deaths

Some international agencies require data on both live births and fetal deaths, but others do not include fetal deaths in their mortality statistics. Therefore, if fetal deaths are included in the national mortality register, they must be easy to identify, so that data include or do not include fetal deaths as requested by the agency to which the data will be delivered.

If it is not clear whether the death certificate relates to a fetal death or a child born alive, refer back to the certifier if possible. If the certifier confirms that it was a case of fetal death, or if other evidence points to a fetal death, then flag the death as a fetal death in the mortality statistics. If no cause of death is stated, code to [KD3B.Z](#) Unspecified time of fetal death, cause not specified.

If the certifier states that the child was born alive but does not report the cause of death, then code to [KD5Z](#) Conditions originating in the perinatal or neonatal period, unspecified.

Refer to [2.25.4.2](#) for the definitions of live birth and fetal death.

2.21.8.5 Developmental anomalies

Conditions classified as Developmental anomalies should be coded as congenital if the duration of the condition indicates that it existed from birth, even if the condition is not specified as congenital on the death certificate. This applies to all conditions for which a specific congenital code is available, whether or not the code is in Chapter 20. Refer to the coding tool for the appropriate code for the condition with the modifier 'congenital'.

Further, the following conditions should be coded as congenital at the ages stated, provided there is no indication that they were acquired after birth:

- Under 1 year of age
 - aneurysm
 - aortic stenosis
 - atresia
 - atrophy of brain
 - cyst of brain
 - deformity
 - displacement of organ
 - ectopia
 - hypoplasia of organ
 - malformation
 - pulmonary stenosis
 - valvular heart disease
- Under 4 weeks of age
 - heart disease NOS
 - hydrocephalus NOS

2.21.8.6 Multiple injuries in the same body region and Injuries involving multiple body regions

In multiple cause coding, do not use codes for multiple injuries of the same body region 'Multiple injuries of specific sites' ([NA00-ND1Z](#)) or codes for [ND30-ND37](#) Injuries involving multiple body regions*, if specific information on the injuries involved is available. Code each injury separately and use as specific injury codes as possible. The information on multiple injuries is obtained in the multiple cause code string as a set of specific injury codes.

2.21.8.7 Complications of surgical and medical care

There is a short list of specific complications not elsewhere classified in Chapter 22, ([NE8Z](#)). Early complications and conditions arising from devices, implants or grafts are coded here. Code late complications and longstanding complications of organ function to the postprocedural section in the appropriate system chapter.

2.21.8.8 Intent of external causes

Undetermined intent

The block for undetermined intent ([PF40-PH8Z](#)) covers events where available information is insufficient to enable a medical or legal authority to make a distinction between unintentional causes, intentional self-harm or assault ([External causes of injury by intent and mechanism](#)). The following cases are included:

- When external causes are reported and the intent could not be determined or is stated as unknown
- When self-inflicted injuries are reported without specification of intent

Note that self-inflicted poisonings reported without specification of intent are assumed to be unintentional and are coded to [PB20-PB36](#) *Unintentional exposure to or harmful effects of substances*.

Note that reporting of the intent may be affected by legal provisions in each country or region - assignment of codes should follow such provisions where appropriate.

Intent not recorded or stated as unknown

Knowledge of intent is necessary for the coding of external causes (chapter 23) and this can usually be inferred from the recorded intent (sometimes called manner of death) if it is not clear from the wording of a stated external cause, or if an injury (chapter 22) is reported without an external cause. Note that 'undetermined' when stated as such is an acceptable intent for this purpose (see above).

However, in some countries the intent may not be reported as part of death certification. If national practice allows, additional information such as a police or coroner's report may be taken into account in the coding process for external causes.

If no information on intent is available, infer the intent from the main injury as follows:

- Any mention of gunshot, shooting, any type of firearm, stab wound, wound from sharp object, hanging, strangulation: Undetermined
- Any injury or harm arising from surgical or medical care: Unintentional
- Any other injury or poisoning: Unintentional For all deaths not involving injury, the default intent is Natural causes.

Default external cause code for an injury, when there is no additional information

As a condition in chapter 22 (injury) cannot be used for underlying cause of death, it is necessary to ensure a code from chapter 23 (external cause of death) is included, either in Part 1 or Part 2. However, if an injury is recorded but there is no mention of an external cause nor are there any other sources of information that may be available according to national practices, then code the external cause using the default codes in the below table.

In applying the default code, please ensure the manner of death (sometime called intent) is taken into account, to ensure the correct default code is applied.

Note: The table below provides default codes to be used only if there is insufficient information to select a more specific code in Chapter 23 considering the contents of the MCCD and any other (additional) sources of information that may be available.

These default codes can be subject to change according to different national situations. Such changes should be based on statistical or epidemiological evidence relating to most likely external cause.

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
[{22}(https://icd.who.int/browse/latest-release/mms/en#435227771) Injury, poisoning or certain other consequences of external causes]	PB6Z Unspecified unintentional cause of morbidity or mortality	PF2Z Assault, unspecified	PD3Z Intentional self-harm, unspecified	PH8Y Other specified injury event of undetermined intent
Open bites	PA7Z Unintentional contact with person, animal or plant, type unspecified	PE1Z Assault by contact with person, animal or plant, type unspecified	PC4Z Intentional self-harm by contact with person, animal or plant, type unspecified	PG6Z Unspecified type of contact with person, animal or plant of undetermined intent
Whiplash	PA5Z Unintentional transport injury event, unspecified	PD9Z Assault by transport injury event, unspecified	PC2Z Intentional self-harm by transport injury event, unspecified	PG4Z Unspecified transport injury event of undetermined intent
Blast injuries	PB55.Z Unintentional exposure to explosion, unspecified	PF15 Assault by exposure to explosion	PD25 Intentional self-harm by explosion or rupture of pressurised materials or object	PH75.Z Exposure to explosion with undetermined intent, unspecified
Nonvenomous bites	PA75 Unintentionally bitten by animal	PE15 Assault by being bitten by animal	PC44 Intentional self-harm by being bitten by animal	PG65 Bitten with undetermined intent by animal

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
Splinter	PA83.2 Unintentionally cut or pierced by other or unspecified sharp object	PE4Z Assault by exposure to unspecified object, not elsewhere classified	PC53.Z Intentional self-harm by being cut or pierced by sharp object, unspecified	PH0Z Exposure to unspecified object, not elsewhere classified, undetermined intent
Foreign body	PB50 Unintentional exposure to foreign body in orifice	PF10 Assault by foreign body in orifice or eye	PD20 Intentional self-harm by foreign body in orifice	PH70 Exposure to foreign body in orifice or eye with undetermined intent
Burns, except acute skin injury due to contact with corrosive substance	PB1Z Unintentional exposure to unspecified thermal mechanism	PE7Z Assault by exposure to unspecified thermal mechanism	PC8Z Intentional self-harm by exposure to unspecified thermal mechanism	PH3Z Exposure to unspecified thermal mechanism with undetermined intent
Acute skin injury due to skin contact with corrosive substance	PB34 Unintentional exposure to or harmful effects of corrosive substances	PE94 Assault by exposure to or harmful effects of corrosive substances	PD04 Intentional self-harm by exposure to or harmful effects of corrosive substances	PH54 Exposure to or harmful effects of undetermined intent of corrosive substances

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
Frostbite	PB16 <i>Unintentional exposure to excessive cold</i>	PE76 <i>Assault by exposure to excessive cold</i>	PC86 <i>Intentional self-harm by exposure to excessive cold</i>	PH36 <i>Exposure to excessive cold with undetermined intent</i>
Drug toxicity	PB28 <i>Unintentional exposure to or harmful effects of other or unspecified drug, medicament or biological substance</i>	PE8Z <i>Assault by exposure to or harmful effects of drugs, medicaments or biological substances, unspecified</i>	PC98 <i>Intentional self-harm by exposure to other or unspecified drug, medicament or biological substance</i>	PH48 <i>Exposure to or harmful effects of undetermined intent of other or unspecified drugs, medicaments or biological substances</i>
Other substance toxicity	PB36 <i>Unintentional exposure to or harmful effects of other or unspecified substances chiefly nonmedicinal as to source</i>	PE95 <i>Assault by exposure to or harmful effects of other or unspecified substances chiefly nonmedicinal as to source</i>	PD05 <i>Intentional self-harm by exposure to or harmful effects of other or unspecified substances chiefly nonmedicinal as to source</i>	PH56 <i>Exposure to or harmful effects of undetermined intent of other or unspecified substances chiefly nonmedicinal as to source</i>

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
Injury or harm from infusion, transfusion or injection	PK81.7 <i>Injection or infusion for therapeutic or diagnostic purposes associated with injury or harm in therapeutic use</i>	N/A	N/A	N/A
Selected injury or harm from medical and surgical procedures	PK8Z <i>Surgical or other medical procedures associated with injury or harm in diagnostic or therapeutic use, unspecified</i>	N/A	N/A	N/A
Injury or harm from pacemaker or cardiac device	PK91.1 <i>Cardiovascular devices associated with injury or harm, therapeutic, nonsurgical or rehabilitative devices</i>	N/A	N/A	N/A

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
Drug toxicity related to medical or surgical care	PL0Z <i>Substances associated with injury or harm in therapeutic use, unspecified</i>	N/A	N/A	N/A
Effects of radiation	PB53 <i>Unintentional exposure to radiation</i>	PF13 <i>Assault by exposure to radiation</i>	PD23 <i>Intentional self-harm by exposure to radiation</i>	PH73 <i>Exposure to radiation with undetermined intent</i>
Effects of heat	PB15 <i>Unintentional exposure to excessive heat</i>	PE75 <i>Assault by exposure to excessive heat</i>	PC85 <i>Intentional self-harm by exposure to excessive heat</i>	PH35 <i>Exposure to excessive heat with undetermined intent</i>
Hypothermia and cold	PB16 <i>Unintentional exposure to excessive cold</i>	PE76 <i>Assault by exposure to excessive cold</i>	PC86 <i>Intentional self-harm by exposure to excessive cold</i>	PH36 <i>Exposure to excessive cold with undetermined intent</i>
Asphyxiation	PB0Z <i>Unintentional threat to breathing by unspecified means</i>	PE6Z <i>Assault by threat to breathing, unspecified</i>	PC7Z <i>Intentional self-harm by threat to breathing, unspecified</i>	PH2Z <i>Threat to breathing with undetermined intent, unspecified</i>

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
Strenuous exercise	PB56 Unintentional exposure to physical overexertion	PF16 Assault by physical overexertion	PD26 Intentional self-harm by physical overexertion	PH76 Physical overexertion with undetermined intent
Hunger	PB57 Unintentional lack of food	PF17 Assault by lack of food	PD27 Intentional self-harm by lack of food	PH77 Lack of food with undetermined intent
Thirst	PB58 Unintentional lack of water	PF18 Assault by lack of water	PD28 Intentional self-harm by lack of water	PH78 Lack of water with undetermined intent
Exhaustion	PB59 Unintentional other specified privation	PF19 Assault by other specified privation	PD29 Intentional self-harm by other specified privation	PH79 Other specified privation with undetermined intent
Other specified deprivation	PB59 Unintentional other specified privation	PF19 Assault by other specified privation	PD29 Intentional self-harm by other specified privation	PH79 Other specified privation with undetermined intent
Lightning	PJ00 Victim of lightning	PF2Y Other specified assault	PD3Y Other specified intentional self-harm	PH8Y Other specified injury event of undetermined intent

Type of injury	Unintentional	Assault	Intentional Self-Harm	Undetermined intent
Drowning	PA9Z <i>Unintentional immersion, submersion or falling into water, unspecified</i>	PE5Z <i>Assault by immersion, submersion or falling into water, unspecified</i>	PC6Z <i>Intentional self-harm by immersion, submersion or falling into water, unspecified</i>	PH1Z <i>Immersion, submersion or falling into water with undetermined intent, unspecified</i>
Vibration	PB6Y Other unintentional cause of morbidity or mortality	PF2Y Other specified assault	PD3Y Other specified intentional self-harm	PH8Y Other specified injury event of undetermined intent
Motion sickness	PB6Y Other unintentional cause of morbidity or mortality	PF2Y Other specified assault	PD3Y Other specified intentional self-harm	PH8Y Other specified injury event of undetermined intent
Electric current	PB51 <i>Unintentional exposure to electric current</i>	PF11 <i>Assault by exposure to electric current</i>	PD21 <i>Intentional self-harm by exposure to electric current</i>	PH71 <i>Exposure to electric current with undetermined intent</i>

If an injury code is selected on an MCCD as the tentative underlying cause of death TUC when no external cause is stated but the manner of death is indicated as war or legal intervention apply the following instructions against the (TUC).

TUC is:

Chapter 22 Injury, poisoning or certain other consequences of external causes

Chapter 22 Injury, poisoning or certain other consequences of external causes

code to:

[PK6Z](#) *Unspecified armed conflict* if Manner of death stated on MCCD is War

[PJ4Z](#) *Legal intervention involving unspecified means* if Manner of death stated on MCCD is Legal intervention

2.21.8.9 Coding of transport injury events

See Section [2.23.19](#) for standard and coding instructions for transport injury events.

1. *Land transport injury events can be “road traffic” or “off-road nontraffic”. Consider it as a road traffic injury event unless one of the following is true, in which case consider it as an off-road nontraffic injury event:*

- - a) Place of occurrence was stated explicitly as off-road or non-traffic
- - b) Place other than a public street or highway is specified
- - c) Only pedestrians or animals were involved

Special vehicles (see [2.23.19.1](#) categories u, v, w and x) are an exception to this rule. If the place of occurrence (on-road or off-road) is not available, and it is unknown whether the special vehicle type primarily operates on-road or off-road in your country or setting, code to categories in “Unintentional land transport injury event unknown whether road traffic or off-road nontraffic.

2. *When unintentional injury involving more than one kind of transport is reported, use the following order of precedence:*
 - aircraft and spacecraft
 - watercraft
 - other modes of transport
3. *When a land transport injury event description does not specify the role of person injured:*
 - *Consider the person injured as a pedestrian, if the person injured is described as crushed, dragged, hit, injured, killed, knocked down, or run over by any vehicle.*
 - *If not, consider the person injured as an occupant or rider of the vehicle mentioned.*
4. *When more than one vehicle is mentioned, do not make any assumption as to which vehicle was occupied by the person injured unless the vehicles are of the same type. Instead, code to the appropriate categories, taking into account the order of precedence given in note 2 above.*

2.21.8.10 Factors influencing health status or contact with health services

This chapter should not be used for international comparison or for primary mortality coding (that is, as the underlying cause of death). Codes in the chapter may be added as multiple cause codes.

2.21.8.11 Infectious agents reported alone on a death certificate

When an infectious agent is reported alone on a death certificate and if no Index Term is found for the infection of the infectious agent, refer to Chapter X and code to the infection of unspecified site of the most detailed parent of the infectious agent.

Example 1

1 (a) Adenovirus [1D90](#)

(b)

(c)

(d)

2

Adenovirus is an infectious agent classified to [XN000](#). Code to [1D90](#) **Adenovirus infection of unspecified site**.

Example 2

1 (a) Staphylococcus aureus [1C41](#)&[XN6BM](#)

(b)

(c)

(d)

2

Staphylococcus aureus is an infectious agent classified to XN6BM. There is no Index Term for infection by staphylococcus aureus. Code to the infection of its most detailed parent Staphylococcus, [1C41](#) **Bacterial infection of unspecified site** and add [XN6BM](#) to identify the specific infectious agent reported.

Example 3

1 (a) Escherichia Coli [1C41](#)&[XN6P4](#)

(b)

(c)

(d)

2

Escherichia Coli is an infectious agent classified to XN6P4. There is no Index Term for infection by 'Escherichia Coli', and nor for its parents [XN4WC](#) Escherichia and [XN5PZ](#) Gram Negative Bacteria. Code to the infection of its most detailed parent Bacteria at [1C41](#) Bacterial infection of unspecified site, and add XN6P4 to identify the specific infectious agent reported.

2.21.8.12 Coding of sudden unexplained deaths

2.21.8.13 Special instructions on Dementia due to Alzheimer disease with early or late onset

If early onset or late onset is explicitly stated, assign the appropriate ICD-11 code accordingly.

If the type of onset is not explicitly stated, determine the appropriate category using the age at death and duration of disease:

- If the age or the calculated age of onset is less than 65 years, code to early onset ([6D80.0](#)).
- If the calculated age of onset is 65 years or over, code to late onset ([6D80.1](#)).

When the age at death is 65 years or over and the duration is unknown or unstated, do not assume the age of onset, and code to [6D80.Z](#) following the text.

Example

- 1 (a) Alzheimer dementia [6D80.0](#) (10 years)
(b)
(c)
(d)
- 2

The age at death was 71 and the duration was 10 years (Time interval from onset to death). As the age of onset of the Alzheimer dementia is 61 years which is less than 65, code to [6D80.0](#) Dementia due to Alzheimer disease with early onset.

2.21.8.14 Pathological fracture

Consider a [Fracture codes](#) as pathological when:

	is reported next to or as the cause of the fracture	code to: FB80.B
cyst of Bone (FB80.6 , FB80.Y)		
hyperparathyroidism 5A51		
infection (1A60 Congenital syphilis, 1A62.01 , 1A62.21 , 1A62.22 , 1A62 , 1B12.40 , 1C1D.2)		
osteogenesis imperfecta (LD24.K0)		
osteolysis (FB86.2)		
osteomyelitis (FB84.Z)		
osteoporosis (FB83.1)		
neoplasms [{02}(https://icd.who.int/browse/latest-release/mms/en#1630407678)]		

2.21.8.15 Conditions that may have either a traumatic origin or a nontraumatic origin

There are conditions that have either a traumatic or a non-traumatic origin (see examples in Table 1). This section is to guide assigning a code to such condition.

First, when such condition is specified as 'traumatic' or 'non-traumatic' assign the traumatic or non-traumatic category in the classification, regardless of other information on the certificate including the manner of death indicated.

When the condition is not specified as either 'traumatic' or 'non-traumatic', first assign a code to the condition following the index. Next use other information on the certificate to infer the origin. This other information is usually a textual statement reported as a) other conditions reported in a sequence causing the said condition or b) an external cause in either Part 1, Part 2 or in Frame B as a description on how the external cause occurred; or c) the manner of death selected (see Table 2). Follow instructions below to decide when to code the condition to a traumatic or nontraumatic category, except otherwise instructed in the reference guide (e.g. 2.21.8.14 Pathological fracture [2.21.8.14](#)).